

VPN NETWORK

CHEAT SHEET

— PEPTIDE GUIDE —

NOOR LABS
*Research.
Innovate.
Elevate.*

PEPTIDE CORE SYSTEM
STATUS: OPTIMAL

KPV	300mcg
BPC-157	300mcg
TB-500	300mcg
GHR-CU	2mg

**CHIEF RESEARCHER
GLOWRILLA
NOOR LABS**

**SCIENCE
IS OUR
ADDICTION**

**TODAY'S MISSION:
TRANSCENDING HUMAN LIMITS**

- ✓ RESEARCH
- ✓ SYNTHESIZE
- ✓ TEST
- ✓ ANALYZE
- ✓ EVOLVE

**RESULTS OR
IRRELEVANCE**

**PEPTIDES
CHANGE BODIES
KNOWLEDGE
CHANGES WORLDS**

EXPERIMENT LOG
**HUMAN POTENTIAL
IS INFINITE**

KPV 300mcg NOOR LABS
BPC-157 300mcg NOOR LABS
TB-500 300mcg NOOR LABS
GHR-CU 2mg NOOR LABS

**RESEARCH
PURPOSES ONLY**

**NOT FOR HUMAN
CONSUMPTION**

**STORE REFRIGERATED
2°C - 8°C (36°F - 46°F)**

GLP Medication Safety: Important Rule-Outs and Cautions



What to rule out, who should avoid them, and who needs extra caution

1 Vision changes: rule out diabetic retinopathy

- GLP medications do not directly cause blindness
- If vision changes happen, rule out diabetic retinopathy first
- Rapid blood sugar improvement can temporarily worsen eye symptoms in some people
- Anyone with diabetes and eye symptoms should get an eye exam



2 Who should avoid these medications

- Avoid if you have a personal or family history of medullary thyroid carcinoma
- Avoid if you have MEN 2, Multiple Endocrine Neoplasia type 2
- This warning applies to the specific thyroid cancer risk listed for these medications



3 Pancreatitis caution

- GLP medications do not clearly cause pancreatitis in most people
- But if someone is already susceptible, the risk may be higher
- Use extra caution if you have a history of pancreatitis or pancreatic problems
- New severe upper belly pain that goes to the back needs urgent medical evaluation



4 Key takeaway

- Rule out diabetic retinopathy if vision changes occur
- Avoid GLP medications with medullary thyroid carcinoma or MEN 2
- Pancreatitis is more about susceptibility and history than a direct effect in everyone
- Talk to your clinician if you have warning signs or high-risk history



Educational reminder only, not personal medical advice.

Warning Signs to Watch For

While using Tirz GLP MEDICATIONS

Educational guide, not a diagnosis

1 Common, watch and monitor

- Mild nausea
- Less appetite
- Mild constipation or diarrhea
- Mild reflux or stomach upset
- Temporary fatigue
- Small injection-site irritation



3 Urgent or ER warning signs

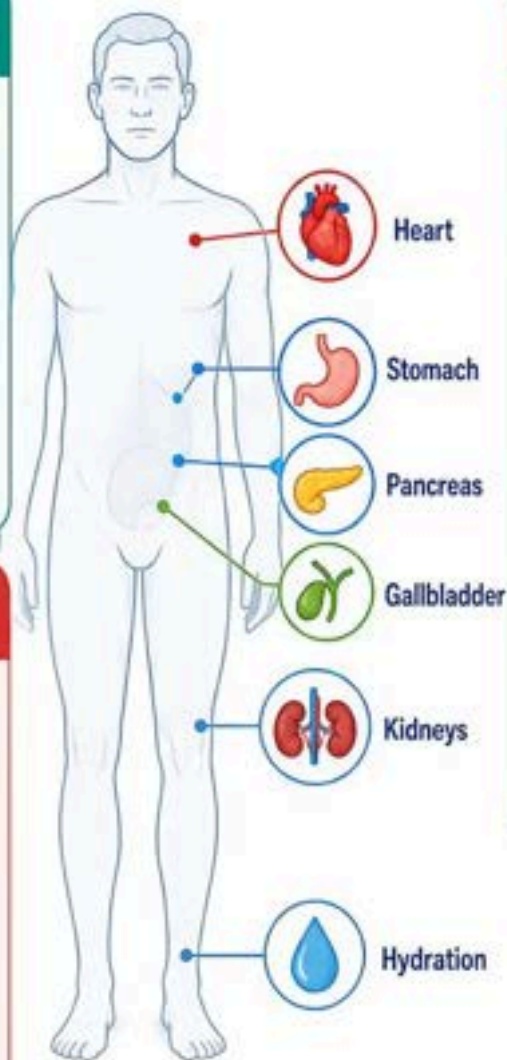
- Severe stomach pain, especially to the back
- Right upper belly pain, fever, yellow eyes, dark urine
- Chest pain, shortness of breath, fainting
- Face or throat swelling, trouble breathing
- Severe weakness, confusion, or signs of low blood sugar
- Bloating belly with no bowel movement, repeated vomiting



**GET HELP FAST.
DON'T WAIT.**

2 Call your clinician soon

- Vomiting or diarrhea lasting over 24 hours
- Dizziness, dry mouth, dark urine, not peeing much
- Constipation getting worse
- Resting heart rate staying high
- Trouble eating enough protein or fluids
- Symptoms getting worse after dose increase



What your body may be warning you about



Dehydration



Pancreas irritation



Gallbladder trouble



Bowel slowdown



Low blood sugar



Kidney stress



Go low and slow, hydrate, use electrolytes, keep protein up, and get help fast if red-flag symptoms appear.

WHY NO ONE SHOULD FEEL GUILTY ABOUT USING GLP MEDICATIONS

You are not weak. You are not cheating. You are treating real biology.

SOME PEOPLE STRUGGLE WITH FOOD NOISE



- ✗ Constant thoughts about food
- ✗ Cravings that won't turn off
- ✗ Never feeling full or satisfied
- ✗ Emotional & compulsive eating

You can't out-think a biological signal. That's why it feels impossible to stop.

WHY MINDSET & BEHAVIOR MATTER BEFORE SURGERY

Before gastric bypass, lap band, or any weight loss surgery...

- ✓ You must address the mental, emotional, and behavioral side.
- ✓ If you don't, you may stretch your stomach again.
- ✓ The tool works best when the mind is prepared.
- ✓ Build habits. Build awareness. Build a new relationship with food.



METABOLIC DYSFUNCTION IS REAL

For many people, it's not just "calories in, calories out." It's biology.



This is where GLP medications can help break the cycle.

GLP MEDICATIONS / PEPTIDES: TOOLS, NOT MAGIC



Semaglutide
Tirzepatide
Retatrutide
& future therapies

THEY HELP BY:

- ✓ Reducing food noise
- ✓ Improving satiety signals
- ✓ Slowing gastric emptying
- ✓ Improving insulin response
- ✓ Helping break the inflammation & insulin resistance cycle

THEY DO NOT:

- ✗ Do the work for you
- ✗ Burn calories by magic
- ✗ Replace healthy habits

They simply make it easier to create the CALORIE DEFICIT.

YOU STILL MUST:

- ✓ Eat enough protein
- ✓ Lift weights / resistance train
- ✓ Move your body
- ✓ Prioritize sleep
- ✓ Manage stress



If you don't feed your body properly, you can lose **MUSCLE** along with fat.

DO NOT FEEL SHAME

Using medication or peptides to support your health is not a moral failing.

You are taking control of your health, not taking the easy way out.



PEPTIDES ARE NOT CHEATING

Peptides (like GLP-1 agonists) work with your body, not against it.

- ✓ They mimic or support hormones your body already makes.
- ✓ They help "unlock" or enhance pathways that are already there.
- ✓ It's like your body's key is broken—these medications help fit the key so your body can do what it's supposed to do.

Body's Natural Key



Broken Lock

STEROIDS ARE DIFFERENT

Steroids are an outside force.

- ✗ They override normal biology.
- ✗ They can shut down your natural hormone production.
- ✗ Your body becomes dependent on an external supply.

This is not the same as supporting a pathway your body already has.



THE DIFFERENCE AT A GLANCE

PEPTIDES / GLP MEDICATIONS

- ✓ Work WITH your body
- ✓ Support natural hormone pathways
- ✓ Help restore normal function
- ✓ Target appetite, satiety, insulin, & more
- ✓ Temporary support while you heal
- ✓ You still have to put in the work

They are a **TOOL** to help you get your life back.

VS

STEROIDS

- ✗ Work AGAINST your body
- ✗ Override natural hormone production
- ✗ Can cause dependence
- ✗ Come with serious long-term risks
- ✗ Shut down your body's natural system

They are an **OUTSIDE FORCE** that can cause more harm than good.

THE GOAL



It's not just about losing weight. It's about

- ✓ Healing your metabolism
- ✓ Reducing inflammation
- ✓ Regaining energy
- ✓ Building muscle
- ✓ Living a long, healthy life

You deserve support.
You deserve options.
You deserve better.



THE BOTTOM LINE

GLP medications are not cheating. They are evidence-based tools that can help you overcome real biological barriers.

Use the tool. Do the work. Change your life.

Your health is your responsibility.
Your healing is your journey.
YOU ARE WORTH IT.



Be patient.
Be consistent.
Be kind to yourself.
Your future self is counting on you.

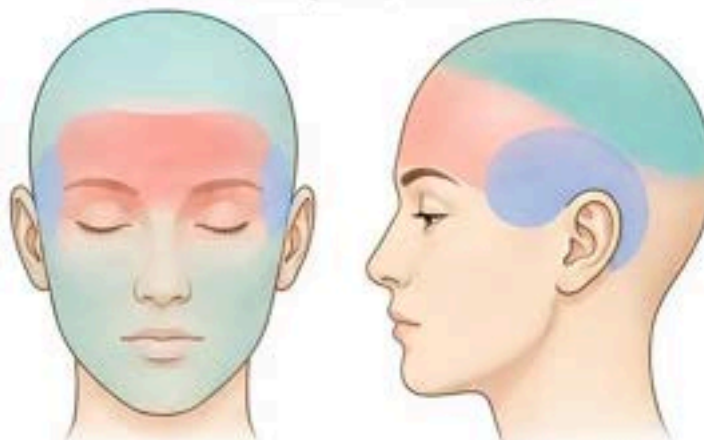
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GLP Headache Area of Pain Compared to Other Headaches

Educational guide, not a diagnosis

PAIN INTENSITY GUIDE

- Severe
- Moderate
- Mild
- Diffuse / Generalized



FRONT VIEW

SIDE VIEW



GLP HEADACHES DO NOT HAVE ONE UNIQUE PAIN SPOT.

Pain is typically diffuse or generalized, especially in the forehead, temples, or whole head.

1 GLP RELATED HEADACHE



- GLP headaches do **NOT** have one unique pain spot.
- Pain is diffuse or generalized, especially forehead, temples, or whole head.
- Often dull or pressure-like
- Can be from dehydration
- Can be from eating too little or low blood sugar
- Can be from electrolyte imbalance
- Sometimes happens after dose increase

2 TENSION HEADACHE



- Tight, pressure, band around head
- Often stress or muscle tension

3 MIGRAINE



- Usually one side
- Throbbing or pulsating
- May include nausea, light sensitivity

4 SINUS HEADACHE



- Face pressure
- Worse when bending forward
- Often with congestion

5 CLUSTER HEADACHE



- Very intense
- Usually around one eye
- May come with tearing or nasal symptoms

RED FLAGS



Sudden worst headache



Headache with chest pain or fainting



Headache with confusion or weakness

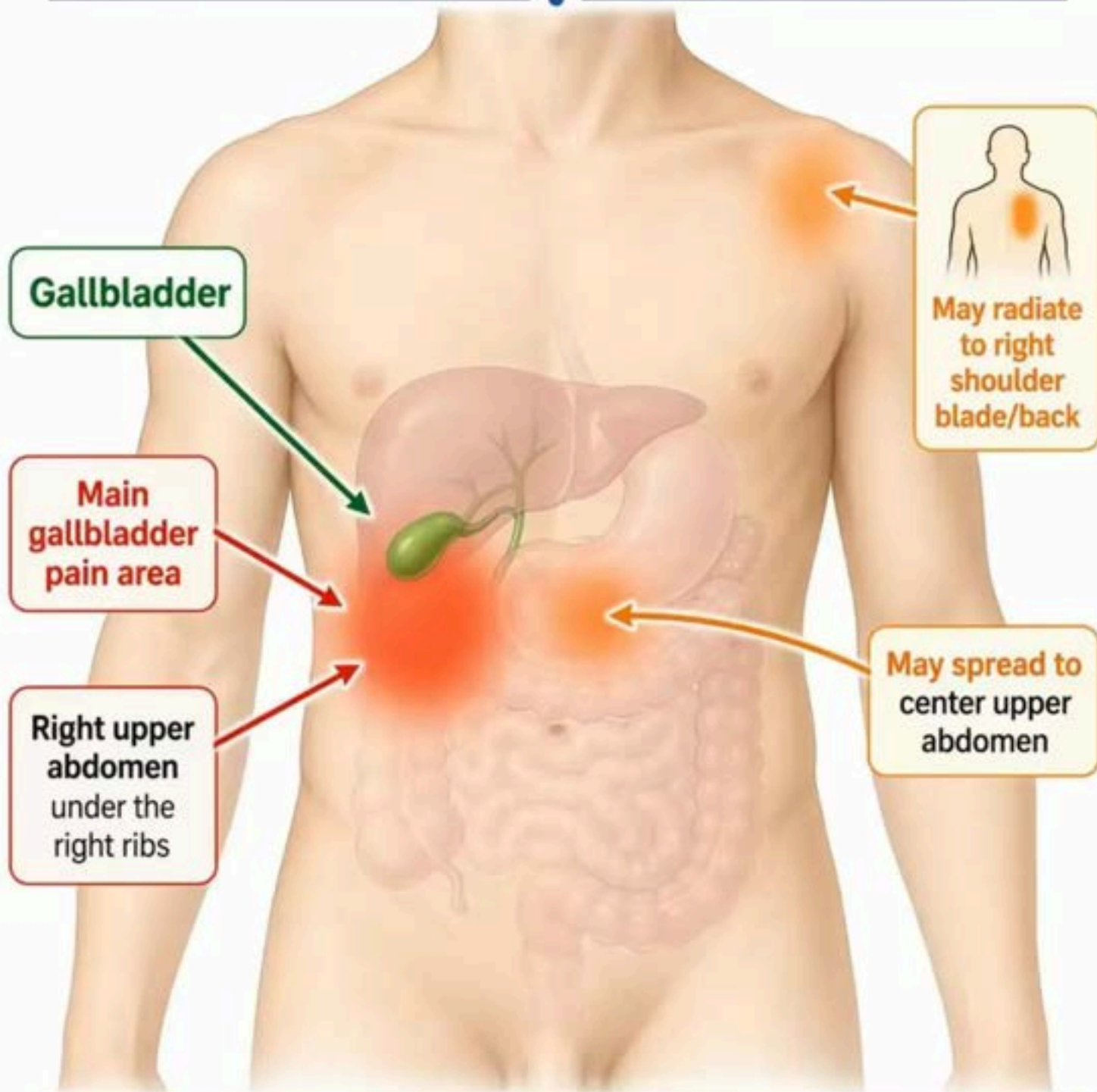


Severe vomiting or dehydration



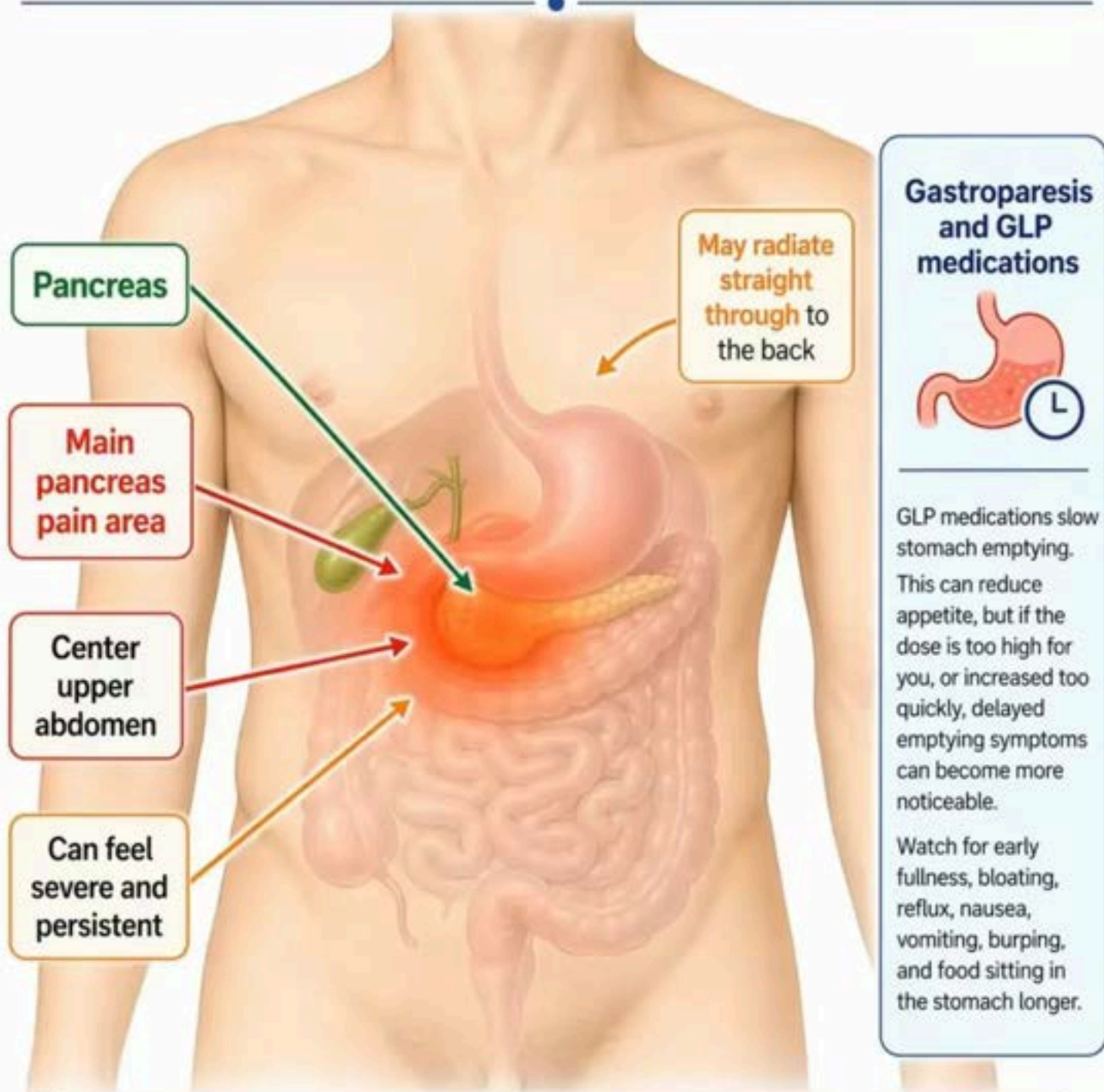
GLP headaches are usually non specific. If the headache is severe, persistent, or comes with red flag symptoms, get medical care.

Gallbladder Pain Area



Important for people on GLP medications: watch for right upper abdominal pain, nausea, vomiting, or pain after fatty meals.

Pancreas Pain Area



Important for people on GLP medications: watch for severe persistent upper abdominal pain, nausea, vomiting, or pain that goes through to the back.

FAT LOSS CAN MOBILIZE TOXINS

Here's what really happens.

Fat cells can store fat-soluble pollutants (toxins) from our environment. When you lose weight, these stored toxins can be released into your bloodstream.



“

When fat cells shrink, they release stored toxins into your bloodstream.

– Yvonne Burkart, PhD

✓ THE FACTS (PROVEN)

Research shows that weight loss can increase the levels of certain fat-soluble pollutants in the blood, including PCBs and organochlorine pesticides.

- Studies show increases of about 2–4% in blood levels per kilogram of weight lost.
- This is temporary and part of your body's natural detox process.

Sources: Meijer et al., 2017, Environmental Int.
Wolff et al., 2000, Int. J. Obes.

WHAT HAPPENS WHEN YOU LOSE WEIGHT



POTENTIAL TEMPORARY EFFECTS



These are usually temporary and a sign that your body is clearing out stored substances.



Detox symptoms are temporary.
Your body is designed to heal.
Support your body and be patient.

SUPPORT YOUR BODY'S NATURAL DETOX

- Stay hydrated**
Drink plenty of water.
- Eat whole foods**
Focus on vegetables, fruits, healthy fats, and clean protein.
- Support your liver & bile flow**
Eat bitter greens, garlic, onions, beets, artichokes.
- Support elimination**
Fiber, probiotics, regular bowel movements, sweating, sauna.
- Get enough protein**
Helps your body repair and detox.
- Lose weight slowly**
Steady fat loss gives your body time to detox.



This information is for educational purposes and not medical advice. Consult your healthcare provider for personal guidance.

ROTATION INJECTION = GUIDE =

ROTATE SMART. STAY SAFE. GET RESULTS.



WHY ROTATION MATTERS

- ✓ Prevents scar tissue & lumps
- ✓ Improves absorption
- ✓ Reduces irritation & pain
- ✓ Keeps sites healthy long-term
- ✓ Ensures consistent results



NEVER inject in the same spot two days in a row.

AREAS TO AVOID (THE "X" ZONES)



UPPER OUTER QUADRANT

Avoid the upper outer quadrant of the buttock.

SCIATIC NERVE

Avoid the path of the sciatic nerve.



SAFE GLUTE LINE

Draw a line from the posterior superior iliac spine (PSIS) to the greater trochanter.

Inject in the upper outer area ABOVE the line.

WHY AVOID THESE AREAS?

- ✗ Risk of hitting the sciatic nerve (major nerve running down the leg)
- ✗ Higher chance of pain, numbness, or nerve damage
- ✗ More blood vessels in these zones
- ✗ Greater risk of lumps, scarring and inflammation
- ✗ Reduced absorption & effectiveness

INJECTION SITES & HOW TO ROTATE

1. STOMACH (ABDOMEN)



FOUR AREAS

- 1 Upper Left
- 2 Upper Right
- 3 Lower Left
- 4 Lower Right

HOW TO ROTATE

Rotate clockwise with each injection. Move at least 1-2 fingers away from the previous spot.

BEST PRACTICE

Stay 2 inches away from the belly button.

2. ARMS (BACK OF UPPER ARM)



SAFE ZONE

Upper outer tricep area (avoid bicep & elbow)

HOW TO ROTATE

Divide the area into 3 zones (top, middle, bottom). Rotate up and down or side to side across sessions.

Best with pinched or relaxed arm.

3. OUTER THIGHS



SAFE ZONE

Outer (lateral) thigh, middle third.

HOW TO ROTATE

Move around the area in a grid pattern. At least 1-2 fingers away from the previous spot.

Avoid inner thigh (more nerves & vessels).

4. GLUTES (BUTTOCKS)



SAFE ZONE

Upper outer quadrant (see guide above).

HOW TO ROTATE

Use a grid pattern within the safe zone. Move 1-2 inches away from the previous spot.

Always inject ABOVE the safe line.

ROTATION EXAMPLE (SAMPLE SCHEDULE)

STOMACH (4 QUADRANTS)



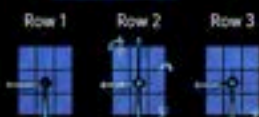
Repeat the cycle.

ARMS (3 ZONES)



Repeat up & down.

OUTER THIGHS (GRID)



Work across the grid.

GLUTES (GRID ABOVE LINE)



Stay above the line.

★ Never reuse the exact same spot until that area has had several days to recover.

GENERAL RULES

- ✓ Use a new needle every time.
- ✓ Clean the area with an alcohol swab.
- ✓ Inject subcutaneously (90° or 45° depending on body fat).
- ✓ Do not inject through clothing.
- ✓ If you feel pain, stop and reassess the area.
- ✓ If a lump forms, avoid the area until fully healed.

NEEDLE SIZE GUIDE (SUBQ)

NEEDLE LENGTH	BODY FAT
4mm	Very lean
6mm	Lean to average
8mm	Average to higher
12.7mm	Higher body fat



SIGNS OF A BAD INJECTION SITE

- ✗ Sharp pain or shooting sensation
- ✗ Numbness or tingling
- ✗ Redness, swelling, or heat
- ✗ Persistent lump or hard knot
- ✗ Bruising or bleeding

★ If any occur, stop using that area.

FINAL REMINDERS

- Rotate Religiously
- Stay Consistent
- Protect Your Sites
- Quality Over Quantity

FOR RESEARCH PURPOSES ONLY

NOT FOR HUMAN CONSUMPTION



STORE REFRIGERATED
2°C - 8°C (36°F - 46°F)

GOOD BACTERIOSTATIC WATER



Clear, colorless, no particles

BAD BACTERIOSTATIC WATER:

HOW TO IDENTIFY IT, RISKS & WHAT TO DO

Even if your peptide or compound is 100% pure, contaminated or poor-quality bacteriostatic water can cause serious problems.

BAD BACTERIOSTATIC WATER



Cloudy, particles discolored

HOW TO IDENTIFY BAD BACTERIOSTATIC WATER



CLOUDINESS / HAZINESS

- Should be crystal clear.
- Cloudy, milky, or hazy = contamination, particles, or improper sterilization.



PARTICLES / FLOATERS

- Visible specks, flakes, or strands are a major red flag.
- Should be completely free of particles.



DISCOLORATION

- Yellow, brown, or off-color = contamination or degradation.
- Should be completely colorless.



STRANGE SMELL

- Should have little to no smell.
- Bad odor = contamination.
- Do not use.



DAMAGED OR TAMPERED VIAL

- Cracked vials, loose seals, or missing caps increase contamination risk.
- Avoid.

SIDE EFFECTS IF BACTERIOSTATIC WATER IS BAD

If contaminated water is injected, you may experience one or more of the following:



INJECTION SITE REACTIONS
Redness, warmth, swelling, pain, tenderness, or itching at the site.



LUMPS / NODULES / ABSCESSES
Hard lumps, knots under the skin, or abscess formation.



SYSTEMIC SYMPTOMS
Fever, chills, body aches, fatigue, or feeling "flu-like".



NAUSEA / HEADACHE / FATIGUE
General malaise, nausea, headache, or dizziness.



SEVERE INFECTION (RARE BUT SERIOUS)
Cellulitis, sepsis, or bloodstream infection. Seek emergency care immediately.



If you develop fever, spreading redness, severe pain, or feel very unwell — seek medical attention right away.

IF YOU HAVE NONE OF THESE SYMPTOMS...



YOU ARE VERY LIKELY FINE.

- No redness, swelling, or pain at injection sites
- No lumps, nodules, or abscesses
- No fever, chills, or flu-like symptoms
- No nausea, fatigue, or unusual reactions
- Clear solution with no particles



This strongly suggests your bacteriostatic water is safe and sterile enough for use.

Many people have used quality bacteriostatic water for years without any issues.

 The key is how your body responds. If you consistently feel good with no adverse reactions, that is the best real-world confirmation.

IF SOMETHING HAPPENS AND YOU GO TO THE ER, WHAT WILL THEY DO?

1. EVALUATION	2. TESTS	3. TREATMENT	4. FURTHER CARE	5. PREVENTION ADVICE
				
They will ask about your symptoms, injections, medications, and when symptoms started. They will check your vitals and examine you.	Blood tests (CBC, inflammatory markers), blood cultures (if infection is suspected), and possibly imaging (ultrasound/CT) if abscess is present.	They may start IV antibiotics if infection is suspected. Pain or fever medication may be given.	If an abscess is present, they may drain it. You may be admitted if the infection is moderate to severe.	They will advise you to stop using the product, monitor symptoms, and follow up with your doctor.

The goal is to identify infection early, treat it, and prevent it from becoming severe.

BEST PRACTICES TO STAY SAFE

-  Buy from reputable, tested sources only. Avoid unknown or sketchy sellers.
-  Check expiration dates and batch/lot numbers. Avoid expired or relabeled products.
-  Always inspect before use. Look for clarity, no particles, and intact seals.
-  Use sterile technique. Clean hands, alcohol swabs, sterile needles, and proper sterilized syringes.
-  Store properly. Keep in a cool, dark place. Do not freeze.

IMPORTANT NOTES

- Bacteriostatic water contains benzyl alcohol to inhibit bacterial growth, but it is not a substitute for proper sterility.
- Once the vial is punctured, contamination risk increases. Use within a reasonable time and keep it clean.
- Not all reactions are from bad water. Peptides like GHK-Cu, BPC-157, or high concentrations can cause irritation.
- When in doubt, don't use it. Your health is more important than any vial.

 **BOTTOM LINE** Bad bacteriostatic water cause noticeable side effects. If you don't have any of them, and the solution is clear, odorless, and particle-free — you are very likely fine. Listen to your body. It's the best indicator.



Bacteriostatic Water Storage Guide



Accurate general storage guidance for bacteriostatic water for injection, USP

1 What it is

Bacteriostatic water for injection, USP is sterile water that contains 0.9% benzyl alcohol as a preservative. It is supplied as a multi-dose vial for repeated withdrawals when proper sterile technique is used.



2 Before puncturing, unopened vial



- Use until the printed expiration date, if the vial is unopened and the seal is intact.
- Store at controlled room temperature: 20 to 25°C, 68 to 77°F.
- Keep away from excessive heat. Do not freeze.
- Do not use if the solution is cloudy, discolored, leaking, or if the seal is damaged.

Tip: many unopened vials may have long shelf life, but always follow the expiration date printed on your vial.



3 After first puncture, opened vial



- Write the date first opened on the vial.
- Discard within 28 days after first puncture, unless the manufacturer states a different date.
- The beyond-use date must never go past the printed expiration date.
- Store at 20 to 25°C, 68 to 77°F, unless your specific product label says otherwise.
- Keep the vial capped, upright, and clean.
- Do not refrigerate or freeze unless the product labeling specifically instructs it.

4 Each time you use it



Wash hands first.



Swab the rubber stopper with 70% isopropyl alcohol and let it dry.



Use a new sterile syringe and needle every time.



Keep the vial out of direct contamination and recap promptly after use.



Discard sooner if the solution becomes cloudy, has particles, changes color, or sterility is in doubt.

5 Quick facts



Unopened vial: store at room temperature, use by printed expiration date.



Opened vial: date it, store at room temperature, discard after 28 days unless label says otherwise.



Best practice: follow the exact manufacturer label for your vial.

i General educational guide. Product-specific labeling and healthcare professional instructions take priority.

Peptide Storage Guide

General storage guidance for lyophilized and reconstituted peptides



1 Before reconstitution, lyophilized peptide powder



Best long-term storage: freezer at -20°C or lower



Typical long-term stability: about 1 to 3 years, often 2 to 3 years when kept dry, sealed, and protected from light



Short-term: refrigerator storage may be acceptable for shorter periods, but freezer is best for long-term preservation



Keep away from heat, moisture, and repeated temperature changes



Store sealed in original box or an opaque container

2 After reconstitution



Store in the refrigerator at 2 to 8°C



Keep upright, protected from light



Best place: back or main body of the fridge, not the door



Use within about 28 days as the conservative rule, especially when mixed with bacteriostatic water; some peptides may differ, so follow supplier guidance if available



Do not leave at room temperature for long periods



Avoid repeated freeze-thaw cycles



Each time you draw a dose



Wash hands



Swab the rubber stopper with alcohol and let it dry



Use a new sterile syringe each time



Return the vial to the refrigerator promptly after use



Keep the vial in a small box, case, or opaque pouch for extra light protection



Brief fridge light exposure from opening the door is usually not a major issue, but protecting from light is still recommended



This is a general guide. Stability varies by peptide, diluent, and manufacturer instructions. Always follow product-specific storage directions when available.



COMMON PROBLEMS PEPTIDES CAN HELP SUPPORT

A Quick Guide

 COMMON CONCERN	 PEPTIDE
 Constant cravings?	Tirzepatide
 Can't get shredded?	Retatrutide
 Slow metabolism?	MOTS-C
 Poor sleep quality?	Epitalon
 Trouble falling asleep?	DSIP
 Dealing w/ pain?	BPC-157
 Systematic healing?	TB-500
 Pale skin?	MT-2
 Gut inflammation?	KPV
 Poor skin quality?	GHK-CU
 Hair loss?	AHK-CU
 Low libido?	PT-141
 Muscle loss?	Tesamorelin
 Immune health?	Thymosin Alpha-1

SUPPORT • RESTORE • THRIVE



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HOW TO RECON CORRECTLY

Reconstitution Instructions



IMPORTANT



USE BAC WATER

Use only Bacteriostatic Water (BAC WATER).

1

ROOM TEMPERATURE

Ensure both vial & water are at room temperature.

2

PLUNGER STEADY

Hold the plunger steady to prevent water from going in too quickly.

3

INJECT ALONG GLASS WALL

- Inject along the glass wall.
- Not directly onto the powder.

4

DO THIS SLOWLY

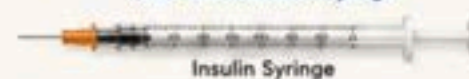
Take your time. Rushing can cause foaming or clumping.

5

USE A 3 mL RECONSTITUTION SYRINGE
NOT AN INSULIN SYRINGE.



3 mL Reconstitution Syringe



Insulin Syringe



EDUCATIONAL PURPOSES ONLY

NOT MEDICAL ADVICE

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Kai+1
PEPTIDE LAB

THE BEST COMBO

Transform From Within

TIRZEPATIDE + GHK-CU + NAD+



SLIMMING BODY

- ✓ Controls appetite
- ✓ Accelerates fat loss
- ✓ Improves metabolism
- ✓ Enhances overall wellness



GLOWING SKIN

- ✓ Boosts collagen
- ✓ Tightens & repairs skin
- ✓ Enhances natural glow
- ✓ Promotes radiance



HIGHER ENERGY

- ✓ Increases cellular energy
- ✓ Improves focus & clarity
- ✓ Reduces fatigue
- ✓ Enhances recovery

Together for a slimmer body, radiant skin, and higher energy.



FOR EDUCATIONAL REFERENCE ONLY. NOT MEDICAL ADVICE.



TIRZEPATIDE

WEEKS	FREQUENCY	DOSAGE			
		MG	FOR 10MG VIAL	FOR 15MG VIAL	FOR 30MG VIAL
1-4 (Loading Phase)	1x per week	2.5	25	25	25
5-8		5	50	50	50
9-12		7.5	75	75	75
13-16		10	100	100	100
17-20		12.5	Vial only contains 10mg	125	125
21+		15		150	150



CYCLE: No cycle. You can stay on it year-round and maintain.



INJECTION SITE: Subcutaneous (abdomen, thigh, back of arm)



RECONSTITUTION SUGGESTIONS:

10mg vial = 1ml Bac water; 15mg vial = 1.5ml Bac water; 30mg vial = 3ml Bac water



IMPORTANT NOTES:

- Store in fridge (2-8 °C).
- Discard 28-30 days after reconstitution or earlier when vial is cloudy.
- If satisfied with low dose, no need to go up a dose. May increase dose after 4 weeks if side effects are tolerated.
- Inject on the same day and same time each week.
- Ensure adequate hydration.
- Caution: 15mg is the maximum dose for this peptide. Do not exceed.



STACKING SUGGESTIONS: AOD-9604, MOTS-c, 5-Amino-1MQ, Cagrilintide, GHK-Cu, BPC-157, TB-500, KLOW Blend, NAD+, SS-31, Epitalon



NOTE: *This is just my personal plan based on my research and what I see online. This is not an expert opinion.*



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SWITCHING GUIDE

TIRZEPATIDE (TIRZ) → RETATRUTIDE (RETA)

CHOOSE BASED ON YOUR TOLERANCE.

No official guideline exists. These are commonly practiced approaches –
choose based on your tolerance.

OPTION 1: DIRECT SWITCH (NO TAPER)

MOST COMMON & PRACTICAL



- ✓ Take your last scheduled Tirzepatide dose
- ✓ Skip Tirzepatide the following week
- ✓ Start Retatrutide at **0.5mg – 1mg**
- ✓ Continue weekly dosing
- ✓ Titrate slowly based on tolerance
- ⚠ Do NOT take Tirz + Retra together



OPTION 2: TAPER → SWITCH

GENTLER APPROACH



- ✓ Gradually reduce Tirzepatide weekly
- ✓ Reach 2.5mg (final dose)
- ✓ Next week → start Retatrutide 0.5mg – 1mg
- ✓ Continue weekly dosing
- ✓ Titrate slowly
- ✓ Easier on nausea, appetite & fatigue



OPTION 3: STOP + WASHOUT

FOR SENSITIVE INDIVIDUALS



- ✓ Stop Tirzepatide completely
- ✓ Wait **7–14 days**
- ✓ Start Retatrutide at **0.5mg – 1mg**
- ✓ Continue weekly dosing
- ✓ Titrate slowly
- ✓ Good for side effect sensitivity
- ✓ Helps “reset” response



IMPORTANT



Do NOT combine
Tirzepatide + Retatrutide



Keep consistent
weekly schedule



Always
titrate slowly



CORE REMINDER

Your response and tolerance are unique.
Adjust gradually, listen to your body, and prioritize long-term success.



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RETATRUTIDE

PROTOCOL GUIDE

RECONSTITUTE. DOSE. TRANSFORM.



RECONSTITUTION GUIDE

VIAL SIZE	ADD BACTERIOSTATIC WATER	
5mg		0.5ml
10mg		1ml
15mg		1.5ml
20mg		2ml



Use only bacteriostatic water for reconstitution.
Swirl gently until fully dissolved. Do not shake.



DOSE RANGE

0.5 – 12mg



FREQUENCY

1x per week (every 7 days)
AM/PM



CYCLE

No strict cycle. Most users transition
to a maintenance dose long-term.



KEY NOTES



Remain at your current dose
until you must increase.
You do not need to increase
unless necessary.



Always dose on the same day
and time of each week.



Most side effects are dose-
dependent. Higher doses over
long durations increase risk.
These effects can be minimized
with proper dosage planning.

SAMPLE DOSING TITRATION GUIDE

WEEKS	FREQUENCY	5MG (0.5ml BW)	10MG (1ml BW)	15MG (1.5ml BW)	20MG (2ml BW)
1	1x / week	5 units	5 units	5 units	5 units
2		10 units	10 units	10 units	10 units
3-6		20 units	20 units	20 units	20 units
7-10		40 units	40 units	40 units	40 units
11-14		60 units	60 units	60 units	60 units
15-18		80 units	80 units	80 units	80 units
19-22		100 units	100 units	100 units	100 units
23+		120 units	120 units	120 units	120 units



EDUCATIONAL PURPOSES ONLY | This content is for informational purposes only and is not
intended as medical advice, diagnosis, or treatment. Consult a qualified healthcare professional.

KAI+1
PEPTIDE LAB

RETATRUTIDE

10mg • BEGINNER GUIDE



WHAT IS IT?

Retatrutide is a novel peptide that acts on three key metabolic hormone pathways to support significant improvements in weight management and metabolic health.

MECHANISM OF ACTION



GLP-1 RECEPTOR AGONISM
Slows gastric emptying, regulates appetite.



GIP RECEPTOR AGONISM
Enhances insulin release, improves fat metabolism.



GLUCAGON RECEPTOR AGONISM
Stimulates energy expenditure, promotes fat breakdown.

KEY BENEFITS

- ✓ Powerful **triple agonist** peptide: GLP-1, GIP, and **glucagon receptors**
- ✓ Optimizes glucose control and fat metabolism
- ✓ Enhanced activity leads to **dramatic fat loss**

POSSIBLE SIDE EFFECTS

- Nausea
- Vomiting
- Headache
- Indigestion
- Constipation or diarrhea
- Abdominal discomfort
- Dehydration



BEST INJECTION SITES



FRONT

BACK

RECONSTITUTION & DOSING



- ✓ **DOSAGE:** 10mg vial + 1 mL BAC Water (100 units)



FIRST TIME USER:

Start with 5 units (0.5mg) – 10 units (1mg) once per week

- ✓ Increase dosing after 4 weeks if needed
- ✓ Use vial within 6–8 weeks once reconstituted

PRECAUTIONS / AVOID IF



- ✗ Pregnant or nursing
- ✗ Chronic kidney or liver disease
- ✗ Gastrointestinal disorders
- ✗ Family history of thyroid cancer



EDUCATIONAL PURPOSES ONLY

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MOTS-C PROTOCOL

	RECONSTITUTION:	10mg → 1mL bacteriostatic water 40mg → 4mL bacteriostatic water
	DOSE RANGE:	2.5-5mg
	FREQUENCY:	1x per day, 3x per week (Mon, Wed, Fri)
	CYCLE:	8 weeks on, 4 weeks off
	NOTES:	- Known to gel. Recommended recon is 1:1 (mg:mL) ratio to avoid gelling. - Best to use a 12mm needle to help avoid injection-site lumps.

PROTOCOL OPTION 1:

Weeks	Frequency	Dose	For 10mg (1mL bw)	For 40mg (4mL bw)
1-4	1x per day, 3x per week (Mon, Wed, Fri)	2.5mg	25 units	25 units
5-8	1x per day, 3x per week (Mon, Wed, Fri)	5mg	50 units	50 units

PROTOCOL OPTION 2:

Weeks	Frequency	Dose	For 10mg (1mL bw)	For 40mg (4mL bw)
1-4	1x per day, 3x per week (Mon, Wed, Fri)	5mg	50 units	50 units
5-8	1 injection, 1x per week (Every Mon)	5mg	50 units	50 units



DISCLAIMER: Educational / informational only.
Not medical advice. Consult a qualified healthcare professional.



BPC-157

= RESEARCH GUIDE =



C₆₂H₉₈N₁₆O₂₂
MW: 1419.55 g/mol

BPC-157 is a stable gastric pentadecapeptide derived from a protein found in human gastric juice. It has potent healing, protective and angiogenic (blood vessel forming) properties.

WHAT IS BPC-157?

- A synthetic peptide derived from a protective protein found in stomach acid.
- Known for its strong healing, regenerative and protective properties.
- Studied for tendon, ligament, muscle, gut and nerve repair.
- Supports angiogenesis (growth of new blood vessels).



POTENTIAL BENEFITS (RESEARCH SUGGESTS)

 Promotes healing of tendons, ligaments, muscles, and soft tissue

 Supports gut health and helps heal ulcers & inflammation

 Supports nerve regeneration and recovery

 Reduces inflammation and oxidative stress

 Stimulates angiogenesis (new blood vessel growth)

 May accelerate recovery and improve overall resilience

RECONSTITUTION GUIDE



Add bacteriostatic water to the vial.

EXAMPLE:
Add 2 mL of bacteriostatic water to a 10 mg vial.

FINAL CONCENTRATION

10 mg ÷ 2 mL =

5 mg/mL



INSULIN SYRINGE GUIDE

- 1 mL = 100 units
- 0.1 mL = 10 units
- 0.01 mL = 1 unit

DOSING GUIDE (BASED ON 5 mg/mL CONCENTRATION)

GOAL	DOSE RANGE	VOLUME (5 mg/mL)	UNITS ON SYRINGE	FREQUENCY	NOTES
 SOFT TISSUE INJURY (TENDONS, LIGAMENTS, MUSCLES)	200 – 500 mcg	0.04 – 0.10 mL	4 – 10 units	1 – 2 x daily	Inject near injury site if possible.
 GUT HEALTH (ULCERS, IBD, LEAKY GUT)	250 – 500 mcg	0.05 – 0.10 mL	5 – 10 units	1 – 2 x daily	Can be injected subcutaneously.
 NERVE RECOVERY	200 – 500 mcg	0.04 – 0.10 mL	4 – 10 units	1 – 2 x daily	Consistency is key. Longer protocols show best results.
 GENERAL RECOVERY / WELLNESS	200 – 400 mcg	0.04 – 0.08 mL	4 – 8 units	1 x daily	Adjust based on response.

★ Start low, go slow. Monitor your body's response and adjust as needed.

HOW TO TAKE

-  Injectable (subcutaneous) – most common in research.
-  Oral / Sublingual – not well studied.
-  Best results with consistent use and proper timing.

STORAGE



- Store lyophilized BPC-157 in the freezer (-20°C / -4°F).
- After reconstitution, store in the refrigerator (2-8°C) and use within 21-30 days.
- Do not freeze reconstituted solution.

SAFETY NOTES



- For research use only.
- Not for human consumption.
- Consult a healthcare professional before starting any peptide protocol.
- Discontinue if adverse reactions occur.

COMMON STACKS (STUDIED TOGETHER)

- TB-500
- AOD-9604
- GHK-Cu
- BPC-157 + TB-500
- NAD+



★ FOR RESEARCH PURPOSES ONLY ★

 FOR RESEARCH PURPOSES ONLY

 NOT FOR HUMAN CONSUMPTION

 STORE REFRIGERATED
2°C – 8°C (36°F – 46°F)



TB-500

= RECONSTITUTION CALCULATOR =

TB-500 (Thymosin Beta-4 Fragment 4-17) is a synthetic peptide derived from thymosin beta-4. It is studied for its potential to promote tissue repair, reduce inflammation, and support cell migration and angiogenesis.



$$C_{34}H_{68}N_{14}O_{10}S$$

MW: 888.05 g/mol

STACK: TB-500 (5 MG)

Reconstituted with 2.0 mL (200 units) of bacteriostatic water.

VIAL CONTENTS

- TB-500: 5 mg
- Total Volume: 2.0 mL (200 units)

CONCENTRATION

- TB-500: 5 mg / 2 mL = 2.5 mg/mL

2.5 mg/mL = 2,500 mcg/mL

2,500 mcg ÷ 100 units = 25 mcg/unit

DOSING CALCULATOR (5 MG)

DOSE	TB-500 (MCG)	TB-500 (UNITS)
500 mcg	500 mcg	20 units
1,000 mcg (1 mg)	1,000 mcg	40 units

500 MCG =

20
UNITS

1 MG =
40
UNITS

DOSING IS BASED ON ACTUAL PEPTIDE CONTENT AND STANDARD CONVERSIONS.
ALWAYS USE STERILE TECHNIQUE AND FOLLOW PROFESSIONAL GUIDANCE.

STACK: TB-500 (10 MG)

Reconstituted with 2.0 mL (200 units) of bacteriostatic water.

VIAL CONTENTS

- TB-500: 10 mg
- Total Volume: 2.0 mL (200 units)

CONCENTRATION

- TB-500: 10 mg / 2 mL = 5 mg/mL

5 mg/mL = 5,000 mcg/mL

5,000 mcg ÷ 100 units = 50 mcg/unit

DOSING CALCULATOR (10 MG)

DOSE	TB-500 (MCG)	TB-500 (UNITS)
500 mcg	500 mcg	10 units
1,000 mcg (1 mg)	1,000 mcg	20 units

500 MCG =

10
UNITS

1 MG =
20
UNITS

DOSING IS BASED ON ACTUAL PEPTIDE CONTENT AND STANDARD CONVERSIONS.
ALWAYS USE STERILE TECHNIQUE AND FOLLOW PROFESSIONAL GUIDANCE.

HOW TO TAKE

- Injectable (subcutaneous or intramuscular).
- Most commonly injected subcutaneously near the injury site.
- Rotate injection sites and use proper sterile technique.

STORAGE



- Store lyophilized TB-500 in the freezer (-20°C / -4°F).
- After reconstitution, store in the refrigerator (2-8°C) and use within 21-30 days.
- Do not freeze reconstituted solution.

SAFETY NOTES



- For research use only.
- Not for human consumption.
- Consult a healthcare professional before starting any peptide protocol.
- Discontinue if adverse reactions occur.

STACKING INFO

TB-500 is commonly stacked with:

- BPC-157
- TB-500 + BPC-157
- ADD-9604
- GHK-Cu
- KPQ
- NAC



FOR RESEARCH PURPOSES ONLY



NOT FOR HUMAN CONSUMPTION



STORE REFRIGERATED
2°C - 8°C (36°F - 46°F)

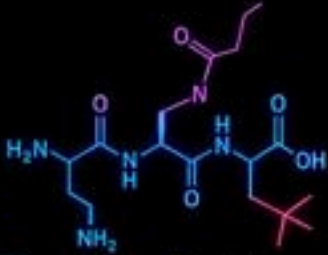




VPN
VERIFIED PEPTIDES
NETWORK

KPV

= RESEARCH GUIDE =



Lys-Pro-Val
 $C_{17}H_{32}N_6O_5$
MW: 384.47 g/mol

KPV (Lys-Pro-Val) is a tripeptide fragment of α -MSH with anti-inflammatory, gut-healing, and immune-modulating properties.

WHAT IS KPV?

- Naturally occurring tripeptide derived from α -MSH
- Known for its anti-inflammatory, antimicrobial, and tissue-repair potential
- Studied for gut, skin, and immune system support



POTENTIAL BENEFITS (RESEARCH SUGGESTS)

 Reduces inflammation

 Modulates immune response

 Supports gut lining integrity

 Supports skin health & may help with dermatitis

 Promotes wound healing & tissue repair

 May aid in recovery and overall wellness

RECONSTITUTION GUIDE



Add bacteriostatic water to the vial.

EXAMPLE:
Add 2 mL of bacteriostatic water to a 10 mg vial.

FINAL CONCENTRATION

10 mg ÷ 2 mL =

5 mg/mL



INSULIN SYRINGE GUIDE

- 1 mL = 100 units
- 0.1 mL = 10 units
- 0.01 mL = 1 unit

DOSING GUIDE (BASED ON 5 mg/mL CONCENTRATION)

USE	DOSE RANGE	VOLUME	UNITS ON SYRINGE	FREQUENCY	NOTES
 GUT HEALTH & INFLAMMATION	500 mcg – 1 mg	0.10 – 0.20 mL	10 – 20 units	1 – 2x daily	Often taken sublingually or orally.
 AUTOIMMUNE / IMMUNE SUPPORT	1 – 2 mg	0.20 – 0.40 mL	20 – 40 units	1 – 2x daily	Adjust based on response and tolerance.
 SKIN HEALTH (DERMATITIS, ACNE)	500 mcg – 1 mg	0.10 – 0.20 mL	10 – 20 units	1 – 2x daily	Topical or oral use studied.
 RECOVERY / WELLNESS	1 – 2 mg	0.20 – 0.40 mL	20 – 40 units	1 – 2x daily	Use consistently for best results.

★ Start low, go slow. Monitor your body's response and adjust as needed.

HOW TO TAKE

- Injectable (subcutaneous) – most common in research.
- Oral / Sublingual – some studies explore oral use.
- Topical – for skin-specific applications.

STORAGE



- Store lyophilized KPV in the freezer (–20°C / –4°F).
- After reconstitution, store in the refrigerator (2–8°C) and use within 21–30 days.
- Do not freeze reconstituted solution.

SAFETY NOTES



- For research use only.
- Not for human consumption.
- Consult a healthcare professional before starting any peptide protocol.
- Discontinue if adverse reactions occur.

COMMON STACKS (STUDIED TOGETHER)

- BPC-157
- TB-500
- Thymosin Alpha-1
- LL-37
- NAD+



★ FOR RESEARCH PURPOSES ONLY ★



FOR RESEARCH PURPOSES ONLY



NOT FOR HUMAN CONSUMPTION

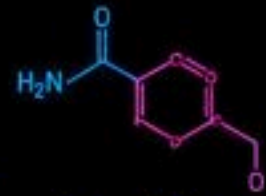


STORE REFRIGERATED
2°C – 8°C (36°F – 46°F)



AOD-9604

= RESEARCH GUIDE =


 $C_{43}H_{66}N_{12}O_8$

MW: 898.08 g/mol

AOD-9604 is a modified fragment of Human Growth Hormone (hGH 176-191) designed to target fat metabolism without affecting blood sugar levels.

WHAT IS AOD-9604?

- A non-GHF fragment that specifically targets fat breakdown.
- Promotes lipolysis (fat breakdown) and inhibits fat re-accumulation.
- Does not significantly affect insulin, blood sugar, or GH/IGF-1 levels.
- Studied for body composition improvement and metabolic support.



POTENTIAL BENEFITS (RESEARCH SUGGESTS)



Promotes fat loss / lipolysis



Does not significantly impact blood sugar levels



Supports body composition improvement



Targets adipose tissue without affecting muscle mass



May enhance metabolic rate



Supports overall metabolic health

RECONSTITUTION GUIDE



Add bacteriostatic water to the vial.



EXAMPLE:

Add 2 mL of bacteriostatic water to a 10 mg vial.

FINAL CONCENTRATION

$$10 \text{ mg} \div 2 \text{ mL} =$$

5 mg/mL



INSULIN SYRINGE GUIDE

- 1 mL = 100 units
- 0.1 mL = 10 units
- 0.01 mL = 1 unit

DOSING GUIDE (BASED ON 5 mg/mL CONCENTRATION)

GOAL	DOSE RANGE	VOLUME (5 mg/mL)	UNITS ON SYRINGE	FREQUENCY	NOTES
FAT LOSS / LIPOLYSIS	200 – 300 mcg	0.04 – 0.06 mL	4 – 6 units	1 × daily	Often fasted or before cardio for best results.
BODY COMPOSITION	300 – 500 mcg	0.06 – 0.10 mL	6 – 10 units	1 × daily	Combine with clean diet and resistance training.
MAINTENANCE / METABOLIC SUPPORT	100 – 200 mcg	0.02 – 0.04 mL	2 – 4 units	1 × daily	Use consistently and monitor progress.

★ Start low, go slow. Monitor your body's response and adjust as needed.

HOW TO TAKE



Injectable (subcutaneous) – most common in research.



Oral / Sublingual – not well studied.



Best results with consistent use and proper diet/training.

STORAGE



- Store lyophilized AOD-9604 in the freezer (-20°C / -4°F).
- After reconstitution, store in the refrigerator (2-8°C) and use within 21-30 days.
- Do not freeze reconstituted solution.

SAFETY NOTES



- For research use only.
- Not for human consumption.
- Consult a healthcare professional before starting any peptide protocol.
- Discontinue if adverse reactions occur.

COMMON STACKS (STUDIED TOGETHER)

- CJC-1295 + Ipamorelin
- TB-500
- BPC-157
- Cardarine (GW-501536)
- Retatrutide



★ FOR RESEARCH PURPOSES ONLY ★



FOR RESEARCH PURPOSES ONLY



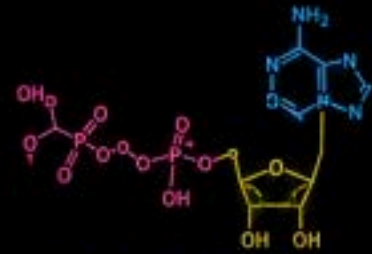
NOT FOR HUMAN CONSUMPTION



STORE REFRIGERATED
2°C – 8°C (36°F – 46°F)

NAD+

RESEARCH GUIDE



$$C_{21}H_{27}N_7O_{14}P_2$$

MW: 663.43 g/mol

WHAT IS NAD+?

NAD+ (Nicotinamide Adenine Dinucleotide) is a coenzyme found in every cell of your body. It plays a critical role in cellular energy production, DNA repair, gene expression, and longevity. NAD+ levels naturally decline with age, stress, poor diet, and lifestyle factors.

KEY BENEFITS (RESEARCH SUGGESTS)



Boosts Cellular Energy
Supports ATP production and mitochondrial function.



Supports DNA Repair
Activates sirtuins and PARPs for cellular repair and protection.



Anti-Aging & Longevity
May slow biological aging and support healthy lifespan extension.



Brain Health & Neuroprotection
Supports cognitive function and may protect neurons from damage.



Metabolic Support
Helps regulate metabolism, insulin sensitivity, and fat oxidation.

WAYS TO INCREASE NAD+ LEVELS



Exercise
Especially HIIT and resistance training.



Intermittent Fasting
Time-restricted eating stimulates NAD+.



Cold Exposure / Heat Hormesis
Stressors that activate cellular defense.



Quality Sleep
Restorative sleep supports NAD+ balance.



Nutrient Dense Diet
Focus on whole foods, polyphenols, B vitamins, and healthy fats.

NAD+ PRECURSORS & BOOSTERS



NR (Nicotinamide Riboside)
Efficiently raises NAD+ levels.



NMN (Nicotinamide Mononucleotide)
Direct NAD+ precursor.



NAD+ (Direct Supplement)
Bypasses conversion for immediate use.



TMG (Trimethylglycine)
Supports methylation and NAD+ synthesis.



Resveratrol / Quercetin
Activates sirtuins and supports NAD+.

NAD+ SUPPLEMENT FORMS



Capsules / Tablets
Convenient and easy to take.



Powder
Flexible dosing and fast absorption.



Sublingual / Liquids
May offer higher bioavailability.



Injectable (Research Use)
For research settings only. Not for human consumption.

DOSAGE GUIDE (GENERAL REFERENCE)

FORM	TYPICAL DOSE	NOTES
NR (Nicotinamide Riboside)	250 – 1000 mg/day	Take in the morning.
NMN (Nicotinamide Mononucleotide)	250 – 1000 mg/day	Take in the morning.
NAD+ (Direct)	100 – 300 mg/day	May be taken any time.
TMG (Trimethylglycine)	500 – 2000 mg/day	Take with food.
Resveratrol	100 – 500 mg/day	Best with a meal.

★ Start low, go slow. Monitor your body's response and adjust as needed.

HOW TO TAKE NAD+ PRECURSORS



Morning
Take in the morning for best energy and circadian alignment.



On Empty Stomach
Take 30–60 min before eating for optimal absorption.



Stay Hydrated
Drink plenty of water throughout the day to support cellular function.

NAD+ STACKING IDEAS

NR or NMN	+	Resveratrol	+	Pterostilbene	=	Sirtuin activation & longevity support
NR or NMN	+	TMG	+	B Complex	=	NAD+ synthesis & methylation
NR or NMN	+	Quercetin	+	Omega-3	=	Inflammation support & cellular health
NR or NMN	+	Magnesium	+	CoQ10	=	Energy production & mitochondrial support

STORAGE



- Store in a cool, dry place.
- Keep away from light and moisture.
- Some forms (especially NMN and NAD+) may benefit from refrigeration.
- Check product label for specific instructions.

SAFETY NOTES



- For research use only.
- Not for human consumption.
- Consult a healthcare professional before starting any new supplement protocol.
- Discontinue if adverse reactions occur.

FINAL REMINDERS



Consistency is key.



Less is more. Start low, go slow.



Research. Track. Adjust.



Quality peptides. Quality results.



RESEARCH PURPOSES ONLY



NOT FOR HUMAN CONSUMPTION



STORE REFRIGERATED
2°C – 8°C (36°F – 46°F)



CJC-1295

+ IPAMORELIN

GROWTH HORMONE

SUPPORT GUIDE



WHAT IS IT?

- CJC-1295 stimulates GHRH receptors
- Ipamorelin stimulates ghrelin receptors
- Works synergistically to support natural GH release
- Commonly used for recovery, sleep, body composition, and wellness research

WHY PEOPLE USE IT


Improved sleep quality


Recovery support


Lean mass support


Fat loss support


Enhanced training recovery


Increased IGF-1 production

BEGINNER PROTOCOL

100 mcg before bed

Daily



BEST FOR:

- ✓ New users
- ✓ Assessing tolerance
- ✓ Sleep support

INTERMEDIATE PROTOCOL

200 mcg daily


100 mcg AM fasted


100 mcg before bed fasted



BEST FOR:

- ✓ Recovery
- ✓ Body composition
- ✓ Consistent GH pulses

ADVANCED PROTOCOL

300 mcg daily


100 mcg AM fasted


100 mcg 5 hours later


100 mcg before bed fasted



BEST FOR:

- ✓ Experienced users
- ✓ Recovery focused protocols
- ✓ Higher frequency GH stimulation

This is your goal protocol.

TIMING GUIDE


MORNING
100 mcg fasted


MIDDAY
100 mcg at least 5 hours after previous dose


NIGHT
100 mcg before bed
Avoid food for 2 to 3 hours beforehand

RESEARCH NOTES

- ✓ Most GH release occurs during sleep
- ✓ Fasted administration is commonly preferred
- ✓ Consistency matters more than high doses
- ✓ Monitor response and adjust conservatively

COMMON CYCLE LENGTH


8 TO 12 WEEKS
Most common cycle


12 TO 24 WEEKS
Extended research cycle

STACKS WELL WITH

- BPC-157
- TB-500
- KPV
- MOTS-c
- NAD+



HOW TO SET IT UP (EXAMPLE WITH 1ML BAC WATER - 100MCG/0.1ML)

1 RECONSTITUTE



Add 1 mL bacteriostatic water to the vial.

2 DRAW



Use an insulin syringe to draw 0.1 mL (100 mcg) per dose.

3 INJECT



Inject subcutaneously (SQ) into clean fatty tissue.

4 ROTATE



Rotate injection sites (abdomen, love handles, thighs).

5 FASTED



Stay fasted before and after doses for best GH response.

6 CONSISTENCY



Be consistent every day. Results come from discipline.

FINAL REMINDERS

- ✓ Start low, go slow.
- ✓ Quality peptides matter.
- ✓ Track sleep, recovery, and body composition.
- ✓ Research, monitor, adjust.

POTENTIAL SIDE EFFECTS (RESEARCH)

- Injection site irritation
- Water retention (temporary)
- Increased hunger
- Headache
- Tingling

Stay hydrated and monitor your response.



FOR RESEARCH PURPOSES ONLY



NOT FOR HUMAN CONSUMPTION



STORE REFRIGERATED
2°C - 8°C (36°F - 46°F)

GHK-Cu INJECTIONS: REACTIONS, TIMELINE & WHEN TO WORRY

GHK-Cu (Copper Peptide) is known to cause local irritation in many people.
Reactions usually improve over time as your body adapts.

1. COMMON INITIAL REACTIONS (Usually Mild & Temporary)



REDNESS

Common and usually fades within hours to 1–3 days.



SMALL BUMPS

May last a few days to 1–2 weeks.



MILD SWELLING

Temporary swelling or firmness.



ITCHING OR TINGLING

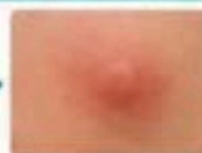
Slight tingling or sensitivity is common with GHK-Cu.

2. ADAPTATION OVER TIME (Many People Improve)



WEEK 1

More redness, bumps, irritation



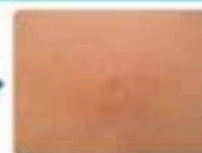
WEEK 2–3

Reactions start to lessen



WEEK 4–6

Minimal redness or bumps



WEEK 6+

Often only very mild or no reaction



AFTER SEVERAL MONTHS

Often no reaction or very mild

Timeframes vary. Full adaptation often occurs in 2–6 weeks.

3. TYPICAL TIMELINE

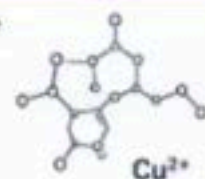


- Redness: Usually resolves in hours to 2–3 days
 - Bumps: May last 3–7 days (sometimes up to 1–2 weeks)
 - Full adaptation: Often 2–6 weeks
- Everyone is different.*

4. WHY GHK-Cu CAUSES REACTIONS



- Copper content can irritate tissue
- Peptide concentration
- pH and solution composition
- Histamine response
- Injecting too shallow
- Repeated use in same area



5. WHEN TO STOP AND SEEK MEDICAL ATTENTION



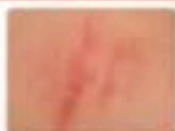
WORSENING
REDNESS OR
SWELLING



HARD, PAINFUL
NODULES



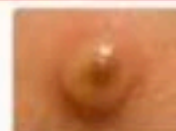
SKIN BREAKDOWN
OR ULCERS



RED STREAKING



FEVER OR
CHILLS



PUS OR
DRAINAGE



Also seek care for: severe pain, rapidly worsening symptoms, allergic reactions (hives, rash, itching, swelling), or feeling unwell.



6. TIPS TO REDUCE REACTIONS



Use proper
sterile technique



Rotate
injection sites



Inject deeper
(subcutaneous)



Use the right
concentration



Inject
slowly



Keep solution
refrigerated



Don't massage
area after
injection

This information is for educational purposes only and is not medical advice.
Consult a healthcare professional for medical concerns.

GHK-Cu INJECTION: WHAT TO EXPECT & WHEN TO WORRY

Most reactions are normal and temporary. Knowing the difference is key.



NOT TO BE CONCERNED

If injected too shallow, GHK-Cu may cause a small blue spot under the skin. This is usually harmless and goes away on its own.

What it looks like:

- ✓ Small blue/gray spot
- ✓ Mild swelling
- ✓ Slight tenderness
- ✓ Improves in a few hours to a couple of days

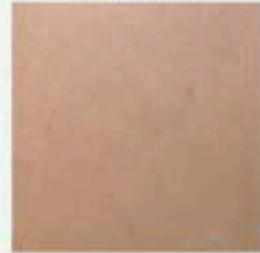
Day 1
Immediately after



Day 2
Improving



Day 3-7
Fading away



What to do:



EXTRA SUPPORT (Optional)

Take an Allergy Pill (Zyrtec)

GHK-Cu can cause a natural histamine response.



Cold Compress
Use for 5–10 minutes at a time to reduce swelling and discomfort.



Why this happens: GHK-Cu can cause a mild histamine response in the area, which is why allergy meds and cold therapy can help. This is your body's normal healing process.



WHEN IT'S A CONCERN

Seek ER care if symptoms get **WORSE** or don't improve.

Watch for these signs of a serious reaction or infection:

1. Excessive Swelling

Spreading or hard lump



Swelling that gets larger or feels hard

2. Severe Redness

Spreading beyond injection site



Redness that spreads, especially with warmth

3. Intense Pain

That gets worse



Throbbing, sharp, or increasing pain

4. Pus or Discharge

Yellow, green, or foul smell



Any pus, unusual odor, or oozing

5. Systemic Symptoms

Body-wide reaction

- Fever or chills
- Nausea or vomiting
- Dizziness
- Trouble breathing
- Hives or rash away from site



GO TO THE ER IMMEDIATELY if you experience any of the above.

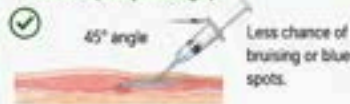
These could indicate infection, allergic reaction, or vascular complications.

WHAT TO DO IN GENERAL

- Always clean the area with alcohol before and after.
- Massage gently – not too hard. Just enough to help absorption.
- Take an allergy pill (Zyrtec) to reduce histamine response (just in case).
- Use a cold compress for swelling or irritation.
- Monitor the area over the next few days.

Injection Depth Matters

Correct (Deep Enough)



Too Shallow



PREVENTION TIPS

- Use the right needle size (usually 30G or 31G).
- Inject at a 45-degree angle into subcutaneous fat, not too shallow.
- Use the right volume per injection (usually 0.1–0.2ml).
- Use sterile technique every time.



BOTTOM LINE

A small blue spot or mild swelling is normal and **NOT** a cause for concern. Care for it gently, support your body, and give it time. Only worry (and go to the ER) if things get worse or you develop systemic symptoms.



Listen to your body.
Be smart, be safe,
and heal confidently.



Kai+1
PEPTIDE LAB

GHK-Cu 100mg

Protocol Guide

Advanced Peptide for Skin Rejuvenation



1 WHAT IS GHK-Cu?

A copper peptide associated with collagen synthesis and skin repair.

2 MAIN BENEFITS

- ✓ Boosts collagen and skin elasticity
- ✓ Supports wound healing
- ✓ Reduces fine lines and improves skin texture and glow
- ✓ Anti-inflammatory and antioxidant effects
- ✓ Mild hair-growth stimulation



Subcutaneous Injection

3 DOSING PROTOCOL (beginner friendly)



Start: 1 mg per day during weeks 1 to 4



Increase: 1.5 mg per day during weeks 5 to 8



Target: 2 mg per day during weeks 9 to 12



Frequency: once daily by subcutaneous injection



Schedule: 5 days on and 2 days off



Cycle: 8 to 12 weeks



Break: 2 to 4 weeks before repeating

4 IMPORTANT NOTES



Do not aspirate



Expect occasional mild redness after injection



Rotate injection sites such as abdomen, thigh, or upper arm



Allow the mixed vial to rest to reduce bubbles

5 RECONSTITUTION GUIDE



Add 3 mL bacteriostatic water



Final concentration approximately 33.3 mg/mL



Store refrigerated



Use within 30 days

6 STACKING (OPTIONAL)



BPC-157
Healing & tissue repair



TB-500
Muscle recovery & endurance



CJC-1295
GH support & recovery



Glutathione
Antioxidant & cellular support



FOR EDUCATIONAL REFERENCE ONLY. NOT MEDICAL ADVICE.



Kai+1
PEPTIDE LAB

GHK-Cu 50mg

Copper Peptide Guide

REGENERATIVE & SKIN SUPPORT



1 WHAT IS GHK-Cu?

GHK-Cu is a naturally occurring peptide made of glycine-histidine-lysine bound with copper, known for regenerative, anti-aging, and tissue-repairing properties.



2 MAIN BENEFITS

- ✓ Promotes firmer, youthful-looking skin
- ✓ Helps reduce wrinkles & fine lines
- ✓ Supports collagen & tissue repair
- ✓ Supports wound healing
- ✓ May improve hair growth

3 DOSING PROTOCOL

- Beginner:** 1 mg per dose, 2–4x per week
- Standard:** 2 mg per dose, 2–3x per week
- Advanced:** up to 5 mg per dose
- Cycle:** rest every 3–4 months once desired goals are achieved

4 HOW TO USE

- Route:** subcutaneous (SubQ)
- Frequency:** 2–4x per week
- Best time:** before bed or morning fasted
- Injection sites:** abdomen, upper hip, thigh; rotate sites

5 RECONSTITUTION GUIDE

- Add 5 mL bac water
- Swirl gently down the vial wall
- Store refrigerated

6 IMPORTANT NOTES

- ! Start low if first time
- ! Store in the refrigerator
- ! Do not let the needle touch the alcohol pad
- ! Discard needle after each use

7 BEST STACKING PEPTIDES

- BPC-157
- TB-500
- CJC-1295
- IGF-1 LR3



FOR EDUCATIONAL REFERENCE ONLY. NOT MEDICAL ADVICE.



TESAMORELIN

— RECONSTITUTION CHART —



STEP 1: RECONSTITUTION



Add **2 mL**
of bacteriostatic
water to the
10 mg vial.

FINAL CONCENTRATION

$$10 \text{ mg} \div 2 \text{ mL} =$$

5 mg/mL

INSULIN SYRINGE GUIDE

- 1 mL = 100 units
- 0.1 mL = 10 units
- 0.01 mL = 1 unit



DOSING CHART (BASED ON 5 mg/mL CONCENTRATION)

WEEK	DOSE (mg)	VOLUME (mL)	UNITS ON INSULIN SYRINGE	VISUAL GUIDE
WEEK 1-2	0.5 mg	0.10 mL	10 units	
WEEK 3	1 mg	0.20 mL	20 units	
WEEK 4-12	2 mg	0.40 mL	40 units	

★ **FOR RESEARCH PURPOSES ONLY** ★



FOR RESEARCH
PURPOSES ONLY



NOT FOR HUMAN
CONSUMPTION



STORE REFRIGERATED
2°C – 8°C (36°F – 46°F)



MOTS-c + SS-31

THE ULTIMATE MITOCHONDRIAL STACK

TRT and Men's Health

TWO PEPTIDES. TWO PATHWAYS. ONE POWERFUL COMBINATION.
MOTS-c SIGNALS THE SYSTEM. SS-31 SUPPORTS THE ENGINE.
TOGETHER THEY TARGET ENERGY FROM TWO ANGLES.

WHAT IS MOTS-c?



A naturally occurring mitochondrial-derived peptide involved in metabolic regulation, insulin sensitivity, glucose handling, and exercise adaptation.


METABOLIC REGULATION


GLUCOSE CONTROL


EXERCISE ADAPTATION


INSULIN SENSITIVITY

WHAT IS SS-31?



A mitochondrial-targeting peptide designed to protect mitochondrial membranes, improve ATP energy production, and reduce oxidative stress.


MITOCHONDRIAL PROTECTION


ATP & ENERGY PRODUCTION


REDUCES OXIDATIVE STRESS


ENHANCED RECOVERY

WHY STACK THEM TOGETHER?

MOTS-c

Improves metabolic signaling & fuel utilization

+

SS-31

Protects & enhances mitochondrial performance

Different pathways. Same goal.
Maximum energy, endurance, recovery, and metabolic optimization.

WHY PEOPLE USE THIS STACK


BETTER DAILY ENERGY


IMPROVED ENDURANCE


ENHANCED RECOVERY


FAT-LOSS SUPPORT


GLUCOSE CONTROL


HEALTHY AGING INTEREST


BETTER TRAINING OUTPUT


REDUCED FATIGUE

SCIENCE STATUS

MOTS-c

- Strong animal research
- Emerging human data
- Growing interest in metabolic health and aging
- Research peptide status

SS-31

- More formal human research than many peptides
- Studied for mitochondrial, cardiac, kidney, and fatigue-related conditions
- Research peptide status

COMMON COMMUNITY DOSING

MOTS-c

10 mg
2 - 3x per week (Subcutaneous)

Most common:
10 mg
1x weekly

SS-31

5 mg daily
(2 - 10 mg daily range used) (Subcutaneous)

Most common:
5 mg daily

CYCLE LENGTH

 **4 WEEKS**
INTRO CYCLE
Assess tolerance and response

 **6-8 WEEKS**
Optimal results for most users

 **10-12+ WEEKS**
EXTENDED CYCLE
Advanced users with reassessment

WHAT USERS OFTEN REPORT

☒ More stable energy

☒ Better stress-resilience

☒ Better cardio tolerance

☒ Improved mitochondrial function

☒ Improved workout recovery

☒ Less "dragging" feeling during the day

☒ Easier calorie deficit phases

BEST USED WHEN BASICS ARE DIALED IN

☒ Nutrition

☒ Stress Management

☒ Sleep

☒ Hormones Optimized (if applicable)

☒ Resistance Training

☒ Consistency

☒ Cardio

REALITY CHECK

 This is not magic. If lifestyle is poor, results are limited. These compounds are performance/longevity tools, not substitutes for discipline.

OPTIMIZE YOUR MITOCHONDRIA. OPTIMIZE YOUR LIFE.

TRT and Men's Health


TRT AND MEN'S HEALTH


RESEARCH PURPOSES ONLY


NOT FOR HUMAN CONSUMPTION


STORE REFRIGERATED
2°C - 8°C (36°F - 46°F)


TRT AND MEN'S HEALTH

BACTERIOSTATIC WATER:

DEMONIZED OVERNIGHT. NOT NEW. NEVER THE PROBLEM.

CAN'T STOP THE PEPTIDE? THEN CONTROL THE WATER.

YOU CAN'T BUY IT FREELY? GOOD. NOW YOU'LL BUY IT FROM US.

ALWAYS BEEN HERE. ALWAYS BEEN USED. NOTHING HAS CHANGED... UNTIL NOW.

DECADES OF SAFE USE IN RESEARCH, COMPOUNDING & PERSONAL USE.

PHARMA INC.

THE FEAR CAMPAIGN:

- ✓ SUDDEN "RISKS"
- ✓ NEW "WARNINGS"
- ✓ HARDER TO FIND
- ✓ SPREAD MISINFORMATION
- ✓ LIMIT SUPPLY
- ✓ RAISE PRICES

FOLLOW THE MONEY.

BACTERIOSTATIC WATER IS DANGEROUS! DON'T USE IT! - TRUST YOUR DOCTOR. TRUST PHARMA.

THE REAL REASON?
GLPs & especially RETA are eating into their billions. They can't ban the peptide. So they target the water.

THEIR NARRATIVE	VS	THE REALITY
✗ "IT'S NOT SAFE" ✗ "USE ONLY OUR PRODUCTS" ✗ "YOU COULD GET SICK" ✗ "IT'S FOR HOSPITALS ONLY" ✗ "UNAPPROVED FOR YOU"		✓ DECADES OF USE ✓ WIDELY USED IN PEPTIDE COMMUNITY ✓ PROPERLY STERILE WHEN HANDLED RIGHT ✓ COSTS PENNIES TO MAKE ✓ THEY JUST DON'T WANT YOU TO GET IT CHEAP

IF THEY CAN'T STOP YOU... THEY'LL PROFIT FROM YOU.

YOU CAN FIND IT: \$5-\$15 PER 30mL VIAL

PHARMA ALTERNATIVE:

- ☠ SAME WATER
- ☠ DIFFERENT LABEL
- ☠ 10X - 20X THE PRICE
- ☠ ONLY DIFFERENCE? THEIR GREED,

THEIR REAL GAME: RETA = BILLIONS
DON'T LOSE MONEY ON THE PEPTIDE. MAKE IT BACK ON THE WATER. \$\$\$

CAN'T BAN THE GUN. SO THEY BAN THE BULLET.

STAY INFORMED. DO YOUR RESEARCH. DON'T LET FEAR MAKE YOU PAY MORE.

KNOWLEDGE IS FREEDOM.

THIS IS NOT MEDICAL ADVICE. THIS IS THE TRUTH. • EDUCATE YOURSELF. • QUESTION EVERYTHING. • THEY DON'T WANT THAT.

**WE ALL HAVE OUR
INNER ANIMAL**
WHEN WE'RE BACKED IN A CORNER BY PHARMA,
**WE RISE.
WE RESEARCH.
WE WIN.**

VPN DATABASE
DATA YOU CAN TRUST.
COMMUNITY YOU CAN COUNT ON.

RUIC
THE RESEARCH FAMILY
TOGETHER WE ARE UNSTOPPABLE.

NOOR LABS
WHERE GLOWRILLA WAS BORN

RESEARCH REX

GLOWRILLA
THEY RESEARCH, NOOR LABS

DATA ORANGE
THEY ANALYZE

✓ WE RESEARCH.
✓ WE TEST.
✓ WE VERIFY.
✓ WE SHARE.
✓ WE HEAL.
✓ WE WIN.

THEIR GAME:
✗ PRICE GOUGING
✗ DENY CARE
✗ LIMIT OPTIONS
✗ SUPPRESS TRUTH
✗ PROTECT PROFITS

OUR GAME:
✓ OPEN DATA
✓ REAL SCIENCE
✓ PEPTIDES FOR PEOPLE
✓ FREEDOM TO HEAL
✓ TOGETHER WE WIN

PHARMA'S FEAR

BACTERIOSTATIC WATER
THEY FEAR IT.
WE UNDERSTAND IT.
WE USE IT.

**KNOWLEDGE
FREEDOM
TRANSPARENCY
HEALTH
FUTURE**
[OUR MISSION]

**WE ARE THE RESEARCH FAMILY. WE ARE THE FUTURE.
WE ARE UNSTOPPABLE.**
RESEARCH. EDUCATE. EMPOWER. ELEVATE.